WHO

Oakridge JMUN 2026

World Health Organization
(WHO)

Agenda: Addressing Mental Health and Addiction Among Young People

Quick Reference Guide
A shorter, simpler version of the full background guide

A Quick Note Before You Start

This guide talks about mental health conditions, drugs, alcohol and, in a few places, suicide. All of it is written for learning and debate. If any part of it feels heavy or reminds you of something in your own life, that is completely okay. Please talk to a parent, a teacher, your school counsellor or any adult you trust. Asking for help is a strength, not a weakness. The Executive Board is also always happy to talk to you during the conference.

About This Quick Guide

The full WHO background guide for Oakridge JMUN 2026 is about 33 pages long. This Quick Reference Guide is a shorter and simpler version for delegates who want the most important facts without reading everything. It follows the same order as the full guide and keeps the key facts, rules, debates and research tips.

Please Read: This Guide Was Made With AI

This summary was written with the help of AI, not by a person. It **may contain mistakes** or leave out details. Please check important facts and numbers in the original background guide and its sources. If anything is confusing, contact the Executive Board.

How to Read This Guide

- **Blue boxes** contain key facts you can use in speeches.
- **Yellow boxes** contain questions to think about, or things to double-check.
- **Green boxes** contain notes about wellbeing and care.
- **Sources** are listed in small text at the bottom of each page. Full links are in the original guide.

A Word About Numbers

Health statistics are *estimates*. Different reports use different years, different age groups and different methods. When you use a number in committee, always say **who** was counted, **when**, and **who** published it. That is what makes a delegate sound trustworthy.

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Rules of Procedure (ROP) in a Nutshell

These are the basic rules that decide how the committee runs. Keep this page handy during debate.

1. Roll Call

Each country says **"Present"** or **"Present and Voting."** If you are "Present and Voting," you cannot abstain on a vote. If you say "Present," you may switch to "Present and Voting" on a later day, but a "Present and Voting" delegate cannot switch back.

2. Setting the Agenda

Delegates make a motion to set the agenda, then debate which topic to discuss first. This motion needs a **simple majority** to pass.

3. Speakers' List

After the committee enters formal debate, delegates join a list to give formal speeches. Raise your placard to add your name. Time limits are set, and the Chair calls speakers in order. You can be on the list only once at a time.

4. Points

Point of Order: fixes a procedural mistake by the Chair.
Point of Personal Privilege: for personal discomfort (for example, the room is too cold or you cannot hear a delegate).
Point of Inquiry: a question about the discussion or a statement in committee.
Point of Parliamentary Inquiry: a question about the Rules of Procedure.

5. Motions

Suspend the Meeting: take a break for caucuses, or end the session.
Close Debate: ends discussion and moves to voting. Needs a **two-thirds majority**.
Table the Topic: postpones debate on a topic. Needs a majority vote.

6. Yields

When you finish early, you can yield your remaining time:
To the Executive Board, who use it as they see fit.
To another delegate (ask their permission first).
To Points of Information from the committee.
To Comments from the committee.

7. Caucuses

Moderated Caucus: the Chair controls who speaks, with shorter and more focused speeches. The topic is a narrower piece of the agenda, chosen by the delegate who proposed it.
Unmoderated Caucus: informal discussion among delegates, without the Board moderating.

8. Draft Resolutions and Amendments

Delegates work together to write **draft resolutions**, which are formal solutions to the issue.
Friendly amendments are accepted by all sponsors without a vote.
Unfriendly amendments must be voted on to be added.

9. Voting

Procedural votes cover motions and amendments. All members must vote.

Substantive votes cover resolutions and unfriendly amendments. Only "Present and Voting" delegates may vote, and abstentions are allowed.

Think About It

Which "Present" choice fits your style? Remember that "Present and Voting" means you must pick yes or no every time.

Source: Rules of Procedure section of the Oakridge JMUN 2026 WHO Background Guide.

About the Committee: The World Health Organization

A Short History

After the Second World War, countries realised that diseases do not stop at borders and that health needed international cooperation. In 1946, countries adopted the **Constitution of the World Health Organization**. It came into force on **7 April 1948**, which is why 7 April is celebrated every year as World Health Day.

WHO is a **specialised agency of the United Nations**. Its headquarters are in Geneva, Switzerland. It works through six regional offices: Africa, the Americas, South-East Asia, Europe, the Eastern Mediterranean and the Western Pacific.

What Does "Health" Mean to WHO?

The first lines of the WHO Constitution say health is "a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity." Notice the word **mental**. Since 1946, WHO has treated mental health as part of health itself, not as a separate or less important issue.

How WHO Is Organised

World Health Assembly
All Member States. Meets every year and makes decisions.

↓

Executive Board
Health experts elected by Member States.

Secretariat
Led by the Director-General. Does the daily work.

↓

Six Regional Offices and Country Offices
Turn global plans into local action.

The Mandate of WHO

A mandate is an organisation's official job description. WHO's objective is "the attainment by all peoples of the highest possible level of health." The jobs in its Constitution most linked to our topic are:

- acting as the directing and coordinating authority on international health work;
- helping governments strengthen their health services when they ask for support;
- proposing conventions, agreements and regulations, and making recommendations;
- fostering activities in the field of mental health;
- promoting and conducting health research, and helping build informed public opinion on health.

What WHO Can and Cannot Do

WHO can: set global guidelines, collect and publish data, give technical help to countries, run joint programmes with other UN agencies, and (through the World Health Assembly) adopt conventions and regulations.
WHO cannot: pass laws inside a country, send police, or force a government to change its policies. Most of what WHO produces are recommendations.
Why this matters for you: a realistic resolution recommends, encourages, invites and requests. It does not order countries around. Keep your clauses within the mandate!

A Real Treaty: the FCTC

The WHO Framework Convention on Tobacco Control was adopted in 2003 and entered into force in 2005. It was the first treaty negotiated under WHO's authority. It is linked to our topic because many young people start using tobacco and nicotine as teenagers.

Membership Update

In 2025 the United States notified its intention to withdraw from WHO, with a stated effective date of 22 January 2026. Membership questions can change, so check the latest official position of your assigned country before the conference.

Sources: WHO Constitution (1946); WHO regional websites; WHO Framework Convention on Tobacco Control website; WHO statement on notification of withdrawal of the United States (24 January 2026).

Introduction to the Agenda

Our agenda is **"Addressing mental health and addiction among young people."** It asks the committee three big questions:

1. How can countries protect young people's mental health **before** problems begin?

2. How can countries stop young people from starting harmful use of alcohol, tobacco, nicotine and drugs, and from unhealthy gaming and social media habits?

3. How can young people who are already struggling get help that is safe, affordable, respectful and easy to reach?

Key Words You Need to Know

Adolescent: a person aged 10 to 19. About one in six people in the world is in this age group.

Mental health condition: a health condition that greatly affects how a person feels, thinks or behaves, such as depression, anxiety or ADHD. It is real and can be treated.

Addiction (dependence): finding it very hard to control use of a substance or activity, and continuing even when it causes harm.

Stigma: shame or negative attitudes attached to a condition. It affects whether teens ask for help.

Protective factor: something that makes a problem less likely, like supportive family, friends and schools.

Mental health: how we think, feel, cope with stress, relate to others and make choices.

Substance use: using alcohol, tobacco, nicotine (including vapes), cannabis or other drugs.

Behavioural addiction: addiction-like habits linked to an activity, not a substance. WHO lists gaming disorder in its disease list (ICD-11).

Risk factor: something that makes a problem more likely, like bullying or violence.

Treatment gap: the difference between people who need care and people who actually get it.

Why This Agenda Matters Right Now

The Big Picture in Numbers



1 in 7 adolescents (10 to 19) has a mental disorder

- About **one in seven (14.3%)** 10 to 19 year olds has a mental health condition, yet most go unrecognised and untreated.
- Mental disorders make up about **15%** of the global burden of disease in this age group.
- **More than 1 billion people** of all ages live with a mental health condition.
- Suicide is the **third leading cause of death** among people aged 15 to 29.
- **23.5%** of 15 to 19 year olds were current drinkers of alcohol in 2019.
- At least **15 million children aged 13 to 15** use e-cigarettes. In countries with data, children are on average **nine times** more likely than adults to vape.

Why Mental Health and Addiction Go Together

WHO explains that risk-taking, such as substance use, often starts in adolescence and can be an unhelpful way to cope with hard feelings. Eating disorders often occur alongside depression, anxiety and substance use disorders, and harmful alcohol use is a risk factor for suicide.

Stress, worry or feeling low → **Using a substance or screen to cope** → **Short relief, then more problems**

Breaking the cycle is the goal. (Real life is more complicated than any diagram.)

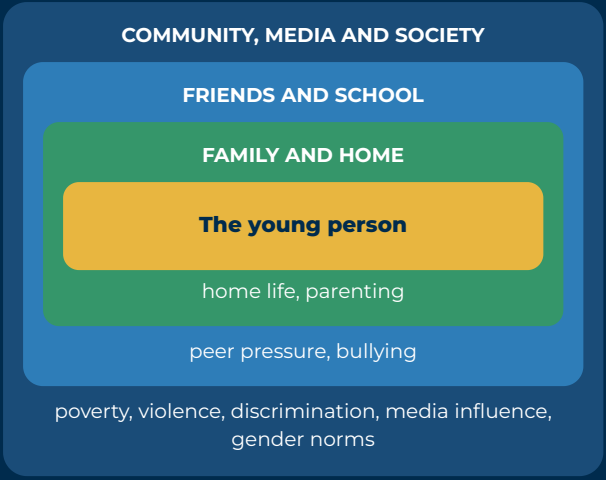
Sources: WHO, Mental health of adolescents fact sheet (2025); WHO news releases of 2 September 2025 and 25 June 2024; WHO tobacco trends report (6 October 2025); WHO Gaming disorder Q&A.

Understanding the Agenda, Part A: Mental Health of Young People

Why Is Adolescence So Important?

WHO calls adolescence "a unique and formative time." Young people go through big physical, emotional and social changes. It is also when many healthy habits are built, like good sleep, regular exercise, problem-solving and learning to manage emotions. Supportive homes, schools and communities help with all of these. Problems left untreated can last into adulthood. In simple words: **helping a 13 year old today also helps the 30 year old they will become.**

What Affects a Young Person's Mental Health?

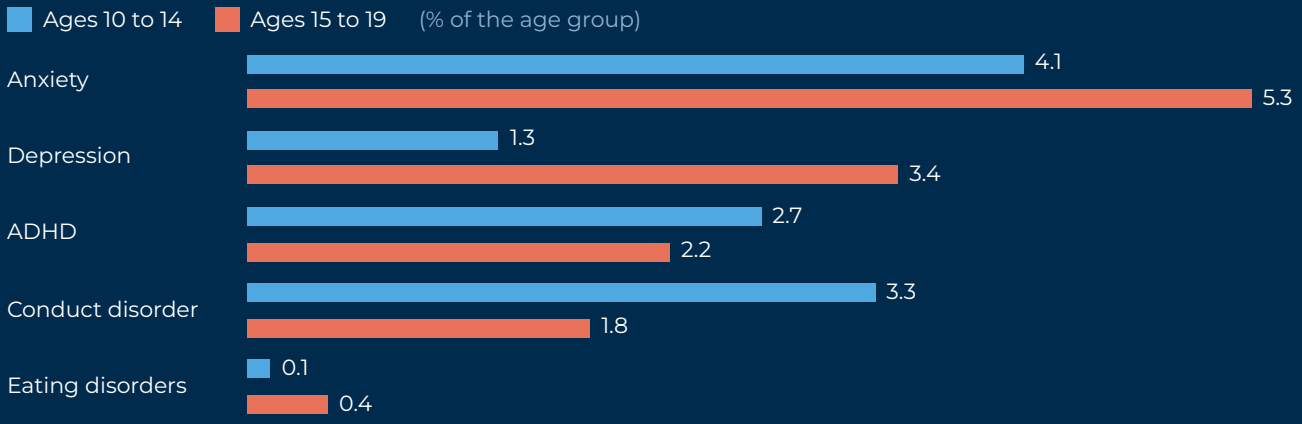


WHO says: the more risk factors young people face, the bigger the effect on their mental health. Risk factors include exposure to adversity, pressure to fit in with peers, exploring one's identity, media influence, gender norms, violence (especially sexual violence and bullying), harsh parenting and serious money problems.

Who May Need Extra Support?

WHO names groups at greater risk because of living conditions, stigma, discrimination, exclusion or lack of services: adolescents in humanitarian and fragile settings; those with chronic illness, autism, an intellectual disability or another neurological condition; pregnant adolescents and adolescent parents; those in early or forced marriages; orphans; and adolescents from minority ethnic or sexual backgrounds or other groups facing discrimination.

Common Mental Health Conditions in Adolescents



- **Emotional disorders.** Anxiety is the most common condition and is more common in older teens. Depression and anxiety can affect attendance and schoolwork, and withdrawing from others can increase loneliness.
- **Behavioural disorders.** ADHD and conduct disorder are more common in younger teens and can affect education.
- **Eating disorders.** They often begin in adolescence or young adulthood, affect girls more often than boys, and often occur with depression, anxiety and substance use disorders.
- **Psychosis.** Symptoms most often begin in late adolescence or early adulthood.

Handled With Care: Suicide and Self-Harm

WHO estimates 727,000 people of all ages died by suicide in 2021. UNICEF reported in 2021 that almost 46,000 children and adolescents aged 10 to 19 die by suicide every year. Risk factors include harmful alcohol use, childhood abuse, stigma against seeking help, barriers to care and access to means. Digital media can either strengthen or weaken prevention. The key message: **suicide is preventable.** WHO's guide LIVE LIFE helps countries with prevention. If this topic affects you, please speak to a trusted adult today.

Sources: WHO, Mental health of adolescents fact sheet (2025), citing the Global Burden of Disease 2021 database; WHO news item (2 September 2025); UNICEF, The State of the World's Children 2021; WHO, LIVE LIFE (2021).

Understanding the Agenda, Part B: Addiction and Substance Use

WHO says young people are especially vulnerable to harmful substance use habits that can last a lifetime. In other words, habits that start at 14 can last for life.

Alcohol

Alcohol is the most widely used substance among young people in many regions. In 2019, **23.5%** of all 15 to 19 year olds worldwide were current drinkers.



Alcohol caused **2.6 million deaths** in 2019 (4.7% of all deaths). The highest share of alcohol-related deaths (13%) was among people aged 20 to 39. WHO's main tools are the **Global Alcohol Action Plan 2022 to 2030** and the **SAFER** package of policies.

Tobacco, Nicotine and E-Cigarettes

In October 2025, WHO published its first global estimate of vaping. More than **100 million people** vape, including at least 86 million adults and at least **15 million children aged 13 to 15**. At the same time, tobacco use overall has fallen: since 2010 the number of tobacco users dropped by 120 million. So the picture is mixed: progress on cigarettes, new worries about newer nicotine products. Many adult smokers had their first cigarette before age 18.

Cannabis and Other Drugs

In 2022, cannabis use among adolescents was **higher** than among adults globally (5.5% compared with 4.4%). The UN Office on Drugs and Crime (UNODC) found that, on average, young people use drugs at least as much as adults. At the launch of the *World Drug Report 2026*, UNODC estimated that **63 million people** have drug use disorders and only about **8%** have access to treatment. Young people are also increasingly seeing cannabis as harmful, an important change in long-term trends.

Gaming and Social Media: "Behavioural" Addictions

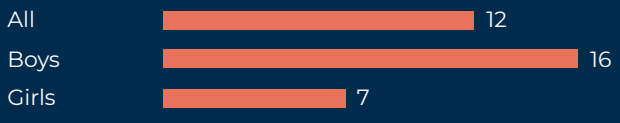
Not every addiction involves a substance. WHO includes **gaming disorder** in ICD-11. It means a pattern of gaming with poor control, gaming taking priority over other interests, and continuing despite harm. WHO adds that it affects only a small share of people who play games.

The HBSC study, run with WHO Europe, surveyed almost 280,000 young people aged 11, 13 and 15 in 44 countries and regions of Europe, Central Asia and Canada in 2022 (*not the whole world*).

Problematic social media use (%)



At risk of problematic gaming, 2022 (%)



The same report found something positive: heavy but non-problematic users reported stronger peer support and social connections. **Technology is not simply "good" or "bad"; how it is used matters.**

Myth or Fact?

"Everyone who plays video games a lot has gaming disorder." MYTH. It affects only a small share of gamers.
"Young people never use drugs as much as adults." MYTH. Teen cannabis use was higher than adult use in 2022.
"Mental health is part of health." FACT. WHO's Constitution has said so since 1946.

Sources: WHO news release (25 June 2024); WHO tobacco trends report (6 October 2025); WHO fact sheet on adolescent mental health (2025); UNODC World Drug Report 2025 and 2026 launch transcript (UN, 26 June 2026); WHO Gaming disorder Q&A; WHO Europe, Teens, screens and mental health (25 September 2024).

Understanding the Agenda, Part C: Why Many Young People Do Not Get Help

Not enough money
Governments spend a median of only about **2%** of health budgets on mental health, unchanged since 2017.

Not enough trained people
The global median is about **13 mental health workers per 100,000 people**. In high-income countries there are about 5.5 child and adolescent psychiatrists per 100,000 children, compared with about 0.1 in other countries.

Low treatment for substance use
For alcohol and drug use disorders together, the share of people in treatment ranged from under 1% to at most 35% (2019). Most of the 145 reporting countries had no specific budget for it, and almost half had no peer support groups.

Stigma and silence, and rich vs poor
Stigma and wrong ideas about treatment add to the gap. In low-income settings fewer than **10%** of people with psychosis get treatment, compared with over **50%** in higher-income countries. Good news: over 80% of countries now include mental health support in emergency responses, up from 39% in 2020.

Think About It

- If a country has very few psychiatrists, who else could be trained to help young people? Teachers? Nurses? Trained peers?
- How can a country reduce stigma so a 13 year old feels comfortable asking for help?
- Is it fair to expect low-income countries to spend more on mental health when they face other urgent health needs? How could the world help?

What the World Has Already Done

A strong resolution builds on what already exists.

1948 WHO Constitution begins; health includes mental well-being.

1989 Convention on the Rights of the Child (Article 24: right to health; Article 33: protect children from illicit drugs).

2003 WHO tobacco treaty (FCTC) adopted; in force 2005.

2010 Global strategy to reduce harmful use of alcohol endorsed.

2013 Comprehensive Mental Health Action Plan (extended to 2030).

2015 Sustainable Development Goals: targets 3.4 (mental health) and 3.5 (substance use).

2019 ICD-11, including gaming disorder, adopted.

2020 WHO guidelines on mental health promotion and prevention for adolescents.

2022 Global Alcohol Action Plan 2022 to 2030.

2025 UN General Assembly declaration on NCDs and mental health (15 December 2025).

Key Frameworks Explained Simply

- **Convention on the Rights of the Child (1989).** Every child has a right to the highest attainable standard of health, and states should protect children from illicit drug use, including through education.
- **Comprehensive Mental Health Action Plan 2013 to 2030.** WHO's main global plan: leadership, community-based services, promotion and prevention, and better information and research.
- **SDG 3.** Target 3.4 promotes mental health and well-being; target 3.5 calls for stronger prevention and treatment of substance abuse and harmful alcohol use.
- **Global Alcohol Action Plan 2022 to 2030.** Guides countries in putting the 2010 strategy into action, with the SAFER policy package.
- **ICD-11 and gaming disorder.** The ICD is the global standard for naming and counting health conditions. Countries consider it when planning public health strategies.
- **Helping Adolescents Thrive (HAT).** A joint WHO and UNICEF effort to strengthen policies that promote adolescent mental health and help prevent self-harm and risky behaviours.
- **mhGAP Intervention Guide 2.0.** Helps non-specialist health workers, such as nurses and general doctors, assess and manage mental health conditions.
- **Mental health in schools.** A WHO Eastern Mediterranean training package for educators.
- **2025 Political Declaration on NCDs and Mental Health.** For the first time, one declaration covers both. It sets a 2030 target of 150 million more people with access to mental health care. It passed 175 in favour, 2 against, 1 abstention. Check how your country voted and why.

Sources: WHO news items (2 September 2025, 25 June 2024, 16 December 2025); UNICEF, State of the World's Children 2021; OHCHR, Convention on the Rights of the Child; UN SDG 3; WHO Action Plan 2013 to 2030; WHO mhGAP 2.0; UN Digital Library, A/RES/80/117.

The Big Debates: Different Perspectives

This is the most important section for your debate. On each issue, countries hold genuinely different views because of different laws, cultures, resources and experiences. The Executive Board does not support any one side. Find out which view **your** country holds and defend it well, while staying open to compromise.

Debate 1: Prevention first or treatment first?

Some argue: Prevention is cheaper and protects more people. School and community programmes can reach many young people before problems start.

Others argue: Struggling young people need help now. Prevention takes years to show results, while untreated conditions can get worse.

Useful fact: WHO's approach includes both prevention and early detection and treatment.

Debate 2: How should countries respond to young people who use drugs?

Some emphasise: Strict laws and strong enforcement, to cut drug supply and send a clear message that drug use is not acceptable.

Others emphasise: Health-centred responses like education, counselling and treatment, so young people are not afraid to seek help.

Useful fact: The drug control treaties and the Convention on the Rights of the Child both apply. UNODC and WHO have published International Standards on Drug Use Prevention and on Treatment of Drug Use Disorders. Many countries combine both approaches.

Debate 3: E-cigarettes and new nicotine products

Some argue: Ban or tightly restrict them (for example flavours and advertising) because so many children use them.

Others argue: They may help some adult smokers switch from cigarettes, so rules should keep them from young people rather than banning them for everyone.

Useful fact: WHO has asked governments to close gaps that let industry target children and to extend tobacco control to new nicotine products. Countries have chosen very different laws.

Debate 4: Social media and gaming: rules or skills?

Some argue: Governments should set minimum ages and hold tech companies responsible for protecting children online.

Others argue: Teaching digital skills, online safety and healthy habits works better. Bans are hard to enforce, and online spaces can also offer real support and friendship.

Useful facts: Australia's social media minimum age law took effect on 10 December 2025. It requires age-restricted platforms to take reasonable steps to stop under-16s from having accounts, with an independent review within two years. WHO Europe recommends school programmes on responsible social media use, online safety, critical thinking and healthy gaming. The two approaches are not opposites, and some countries may choose both.

Debate 5: Who should pay, and who should act?

Some argue: Each country is responsible for its own health system. Governments know their own people, cultures and needs best.

Others argue: Low-income countries need international funding and technical help, because the service gap between rich and poor countries is very large.

Useful fact: WHO data shows large differences in treatment between low-income and higher-income settings.

Debate 6: Medicine, therapy or community?

WHO advises avoiding institutionalisation and over-medicalisation, putting non-medicine approaches first, and respecting children's rights for adolescents with mental health conditions. At the same time, countries differ in how they organise care, the role of families and communities, and how culture shapes the way people talk about mental health.

Think About It

What would a respectful, culturally appropriate system look like in **your** country?

Sources: WHO fact sheet on adolescent mental health (2025); UNODC call for proposals referencing the UNODC-WHO International Standards (July 2024); WHO tobacco trends report (6 October 2025); Australian eSafety Commissioner; WHO Europe, Teens, screens and mental health (2024); WHO news item (2 September 2025).

Speech Guide: What to Avoid and What to Aim For

Speaking time is short, and everyone has already read the agenda. The Executive Board asks every delegate to make each speech **new, clear and useful**.

Please Avoid

1. Repeating the agenda
Do not open by explaining that mental health and addiction are big problems. Everyone knows. Give one line on why it matters to **your country**, then make your point.

2. Repeating other delegates
If your point was already made, say "we agree with..." in a few words, then add something new or pick a different point.

3. Unrealistic solutions
Avoid "end addiction worldwide", "ban all screens for teens" or "WHO will force all countries to...". WHO recommends; it does not command. No huge budgets without a plan for who pays.

4. Numbers with no point
A list of statistics is not an argument. Use one or two numbers, name the source, and explain **what they mean** for your position.

5. Speaking as yourself
Stay in character. "I think..." is not a country's position. Avoid generic speeches that any country could give.

6. Attacking people
Disagree with ideas, not people. Do not mock a delegate or country. These are real people's lives.

Please Aim For

1. Frame your argument in four short steps

Position	Evidence	Why it matters	Solution
What does your country believe?	One fact, with its source.	What it means for your country.	What should be done, by whom?

2. Bring creative and realistic solutions

A good solution is **new** (not the first idea everyone says) and **realistic** (it could really happen). Test each idea: Is it within WHO's mandate (recommend, encourage, invite)? Who does it, who pays, and how will we know it worked? Would it work where there are few doctors or little money?

Ideas from the guide to build on: train teachers, nurses or trained peers to give first support; school programmes on online safety and healthy gaming; young people helping design solutions; richer countries sharing training and technical help; better data where it is missing.

3. Listen, respond and build common ground

Build on, improve or politely challenge other delegates' ideas. Ask sharp Points of Information. Merge clauses with delegates who share your position instead of writing the same one twice.

Weak speech

"Mental health is a very serious problem for young people around the world. Our country wants to end addiction forever, and we ask WHO to make every country ban phones for teenagers."

Strong speech

"Our country has very few psychiatrists. We propose that WHO encourage training teachers and nurses to give first mental health support in schools, using tools like the mhGAP guide, and invite wealthier countries to share training materials."

Before You Speak, Ask Yourself

Am I adding something new? Did I skip the agenda recap? Is my solution realistic and inside WHO's mandate? Do I know who does it, who pays and how it is measured?

Note: this page gives Executive Board expectations for speeches, drawing on WHO's mandate and the research and QARMA sections of the Oakridge JMUN 2026 WHO Background Guide.

How to Research Your Country's Position

Step 1
Find your country on WHO data pages and the WHO country profile.

Step 2
Read your government's health ministry website.

Step 3
Check how your country voted on UN resolutions.

Step 4
Write down 3 facts and 3 solutions.

Research Checklist

- Does my country have a national mental health policy or plan? Does it mention children and adolescents?
- What are my country's laws on the minimum age for buying alcohol and tobacco? Does it regulate e-cigarettes?
- What does my country do about drugs: what are its main laws and programmes?
- Does my country have rules about children and social media or online gaming?
- Does my country have school mental health or counselling programmes?
- How did my country vote on the 2025 Political Declaration on NCDs and Mental Health?
- Is my country a Party to the WHO Framework Convention on Tobacco Control?

Writing Your Position Paper (1 to 2 pages)

1. **Introduction:** why this issue matters to the world and to your country.
2. **Your country's situation:** key facts and what your government already does (with sources!).
3. **Your country's position:** what your country believes should happen.
4. **Proposed solutions:** two or three realistic ideas that fit WHO's mandate.

Golden Rules for Delegates

- Stay in character: speak as your country, not as yourself.
- Every number needs a source. Official sources (WHO, UNICEF, UNODC, UN, your government) are best.
- Be realistic. WHO recommends; it does not command.
- Be kind. You are discussing real people's lives.

Questions a Resolution Must Answer (QARMA)

A good resolution should answer most of these. You do not need to answer them all in one speech.

1. How can schools, families and communities be supported to protect young people's mental health?
2. How can countries with very few mental health workers expand care, for example through trained teachers, nurses or community health workers?
3. What should be done to stop the marketing of alcohol, tobacco and nicotine products to children, offline and online?
4. How can young people who use substances be encouraged to seek help without fear or shame?
5. What is the right balance between protecting young people online and respecting their rights and freedoms?
6. How should countries collect better data on young people's mental health and substance use, especially where data is missing?
7. How can the international community support low-income countries and humanitarian settings?
8. How can young people themselves help design the solutions that affect them?
9. How will progress be measured, and who will report on it?

Where to read more (search these titles): WHO, Mental health of adolescents fact sheet (2025); WHO Comprehensive Mental Health Action Plan 2013 to 2030; WHO tobacco trends report (6 October 2025); WHO Europe, Teens, screens and mental health (2024); UNICEF, State of the World's Children 2021; UNODC World Drug Report 2025. The full guide lists the links.

We look forward to seeing you in committee!

Syed Hudaifah, Pratheek Konda, Archith Ragi
Executive Board, World Health Organization, OakJMUN

Sources: Research checklist, position paper and QARMA sections of the Oakridge JMUN 2026 WHO Background Guide.